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Paper 1 of 4 — MANAGEMENT OF NURSING SERVICES (NSC 413)

Level 500 • First Semester • Handout

LEADERSHIP,

FOLLOWERSHIP, AND

DELEGATION

MANAGEMENT OF NURSING SERVICES (NSC 413)

BY MONISOLA OMISHAKIN DECEMBER, 2021

Leading

- The professional nurse frequently assumes the roles of leader and manager.
- These two roles are linked; that is, managers must have leadership abilities, and leaders often manage, but the two roles differ.
- A leader influences others to work together to accomplish a specific goal.
- Leaders are often visionary; they are informed, articulate, confident, and self-aware.
- Leaders also usually have outstanding interpersonal skills and are excellent listeners and communicators.
- They have initiative and the ability and confidence to innovate change, motivate, facilitate, and mentor others.

Managing

- A manager is an employee of an organization who is given authority, power, and responsibility for planning, organizing, coordinating, and directing the work of others, and for establishing and evaluating standards.
- Managers understand organizational structure and culture. They control human, financial, and material resources.
- Managers set goals, make decisions, and solve problems. They initiate and implement change.

Leadership

- Leadership has been described as the ability to direct or motivate an individual or group to achieve set goals.
- Effective leaders in groups or systems do this by encouraging others to be their best selves as they work collaboratively in the pursuit of common organizational or unit goals.
- Leaders have power, whether it is explicit or

implied

Leadership Qualities

- Most people admire a charismatic, dynamic, enthusiastic, visionary leader who is poised, confident, and self-directed.
- These characteristics define leaders such as Mother Teresa, John Kennedy, Martin Luther King, Gandhi, and others.
- Leaders do need to be comfortable with themselves (i.e., have a positive self-image) and present themselves as role models for followers.
- Ideally, they also have a vision that energizes the group and brings forth the best efforts of members.
- Critical thinkers and responsible decision makers commit high energy to achieving goals and are skilled in enlisting support and cooperation.

Leadership Qualities 2

- Leaders value learning and must be knowledgeable.
- They must be creative in the development and use of scarce resources. For example, nurse leaders can use one another and other healthcare workers as resources, respecting each other's expertise.
- Political awareness is also important for nurse leaders
- Flexibility is a must for all nursing leaders. All nursing functions and roles require flexibility.
- Leadership potential is present in all nurses. With education and practice, these qualities can be developed to the point at which a nurse is skilled in the many behaviours necessary for leadership.

Personal Leadership Skills

The four basic skills needed for nursing leadership listed below are not meant to be all-inclusive, but instead to serve as a basis for development:

- Communication skills: the ability to establish trusting interpersonal relationships with patients, peers, subordinates, and superiors to maximize goal achievement and enhance the personal growth of all participants
- Problem-solving skills: the ability to analyze all sides of a problem, to suspend judgment, to explore multiple options, and to work toward a creative solution.
- Management skills: the ability to direct others toward goal achievement; this ability involves the recognition and fostering of unique talents and skills of others and the ability to match these with necessary tasks, organizational skills, financial skills, and the ability to generate and use resources wisely

□ Self-evaluation skills: the ability to assess honestly one's effectiveness and to accept both praise and criticism; the ability to direct personal professional growth and development

Leadership Theory

□ Early leadership theories focused on what leaders are (trait theories), what leaders do (behavioural theories), and how leaders adapt their leadership style according to the situation (contingency theories).

□ Theories about leadership style describe traits, behaviours, motivations, and choices used by individuals to effectively influence others.

□ Many different styles of leadership are present in contemporary healthcare settings.

□ The complexity and persistent, rapid change drive many of the leadership styles that you will observe.

□ Different styles are applicable in different contexts and with different levels of employees.

CLASSIC LEADERSHIP THEORIES

□ The trait theorists found that leaders often possess specific qualities and abilities including good judgment, decisiveness, knowledge, adaptability, integrity, tact, self-confidence, and cooperativeness.

□ The behaviorists believed that through education, training, and life experiences, leaders develop a particular leadership style.

□ These styles have been characterized as autocratic, democratic, laissez-faire, and bureaucratic.

Leadership Styles

□ 1) Autocratic Leadership- Autocratic leadership, also called directive leadership or authoritarian leadership, involves the leader assuming complete control over the decisions and activities of the group.

□ A nurse with an autocratic personality can be described as "firm, insistent, self-assured, and dominating with or without intent, and keeps at the center of attention".

□ An extremely autocratic leader might make all decisions for workers or followers without considering the followers' ideas or feelings.

Leadership Styles cont'd

□ 2) Democratic Leadership- Democratic leadership,

also called participative leadership, is characterized by a sense of equality among the leader and other participants.

- Decisions and activities are shared.
- Participants are encouraged to develop their skills and strengths within the group.
- The group and leader work together to accomplish mutually set goals and outcomes.

Leadership Styles cont'd

- 3) Laissez-Faire Leadership- In laissez-faire leadership, also called nondirective leadership, the leader relinquishes power to the group, such that an outsider could not identify the leader in the group.
- This approach encourages independent activity by group members.
- This style depends on the strengths of followers to direct the group activities.
- It is most effective when all staff are clinical experts with a deep understanding of both clinical and administrative processes.
- This style is rarely useful because task achievement is difficult when each nurse is working independently, and the staffs on most units and departments have varying levels of clinical maturity.
- However, it can be used effectively when the leader wants a problem to be solved completely by expert staff group members.

Leadership Styles cont'd

- 4) Bureaucratic leadership- The bureaucratic leader does not trust self or others to make decisions.
- Instead, this type of leader relies on the organization's rules, policies, and procedures to direct the group's work efforts.
- Group members are usually dissatisfied with the leader's inflexibility and impersonal relations with them

Leadership Styles cont'd

According to contingency theorists, effective leaders adapt their leadership style to the situation.

5) Situational Leadership- Generally speaking, nurses do best to use the leadership style with which they are most comfortable if that style is effective for the task at hand and for those they lead.

- The nurse's own personality, the group's personality, and

the tasks or objectives to be accomplished should be considered in each leadership situation.

□ Situational leadership theory considers the leader's style, the work group's maturity, and the situation at hand to form a comprehensive approach to management style.

Leadership Styles cont'd

□ Contemporary theorists have described charismatic leaders, transactional leaders, transformational leaders, and shared leadership.

A charismatic leader is rare and is characterized by having an emotional relationship with the group members.

□ The charming personality of the leader evokes strong feelings of commitment to both the leader and the leader's cause and beliefs.

□ When this type of nurse leader speaks to a group, nurses feel inspired and motivated to do whatever they can to meet the leader's expectations.

□ The followers of a charismatic leader often overcome extreme hardship to achieve the group's goals because of faith in the leader.

Leadership Styles cont'd

The transactional leader has a relationship with followers based on an exchange for some resource valued by the follower.

□ These incentives are used to promote loyalty and performance.

□ For example, to ensure adequate staffing on the night shift, the nurse manager entices a staff nurse to work the night shift in exchange for a weekend shift off.

□ The transactional leader represents the traditional manager, focused on the day-to-day tasks of achieving organizational goals, and understanding and meeting the needs of the group.

Leadership Styles cont'd

□ Transformational Leadership- Transformational leadership can create revolutionary change.

□ Often described as charismatic, these leaders are unique in their ability to inspire and motivate others.

□ They create intellectually stimulating practice environments and challenge themselves and others to grow personally and professionally and to learn.

- Gifted in creating a common vision, they demonstrate passion for their vision and keep others similarly focused.
- They communicate honestly and openly and can express emotions as well as ideas as they share themselves with others.
- They show concern and care for others and are willing to take risks.
- They pay attention to process as well as outcomes

Leadership Styles cont'd

- Quantum Leadership- In the new “quantum age,” change is conceived as dynamic, ever-present, and continually unfolding.
- We are forced to experience the change at the same time as we perceive it, with little or no opportunity to definitively and laboriously plan and manage it.
- Characteristics of quantum leadership and behaviours of healthcare leaders who must confront this dramatic social transformation:
 - 1. Healthcare leaders must be able to communicate to others their vision of the future and bring energy and commitment to the reformation of the system.
 - 2. They must be relentless communicators, challenging current ways of thinking and doing.
 - 3. They must openly seek critiques of their work.
 - 4. They must encourage all stakeholders to continually examine the appropriateness of current work rituals and routines and determine what should be retained and what should be left behind as no longer relevant.
 - 5. They must raise questions about the efficacy and effectiveness of current work routines and whether they meet new and emerging expectations and reality.

Shared leadership

- Shared leadership recognizes that a professional workforce is made up of many leaders.
- No one person is considered to have knowledge or ability beyond that of other members of the work group.
- Appropriate leadership is thought to emerge in relation to the challenges that confront the work group.
- Examples of shared leadership in nursing are self-directed work teams, co-leadership, and shared governance.
- Shared governance is a method that aims to distribute decision making among a group of people.
- It provides structure by articulating a mechanism for advocacy and influence of the staff nurse through all levels of nursing. It empowers ownership of quality nursing practice and influences the work environment and administrative functions

Management & Leadership

- Leadership and management are related phenomena but they are not the same.

□ It is important to Understanding the Theory of Leading, Following, and Managing realize that (a) not all individuals in management positions are necessarily leaders, and (b) leadership is not necessarily tied to a position of authority.

□ While only those in management positions are expected to be managers, leadership can and needs to be exercised by each of us wherever we may be.

□ In other words, even though an individual does not hold a management position, she can still be a leader on a clinical unit, in an institution, in her community, or in the profession as a whole.

Management & Leadership cont'd

Leaders and managers are very different Many individuals are able to kinds of people: they differ in their motivations, function as both leaders and in their personal history, and in how they think managers simultaneously and act; they differ in their orientation toward effectively.

goals, work, human relations, and themselves;

and they differ in their worldviews

Indeed, our practice world is

greatly enhanced when

For example, leaders are creative, leaders are able to manage innovative, and risk-takers; managers and managers are able to often are more concerned with lead.

maintaining the status quo and taking few risks.

All nurses need to lead and

In addition, managers often have a short- manage effectively in range perspective and are concerned patient care settings in about the “bottom line,” whereas leaders order to accomplish tasks have a long-range, visionary perspective and achieve maximum care and are concerned about moving toward quality. realization of that vision.

Principles of effective leadership

1) Vision is a mental image of a possible and desirable future state.

□ Leaders transform visions into realistic goals and communicate their visions to others who accept them as their own.

2) Influence is an informal strategy used to gain the cooperation of others without exercising formal authority. Influence is exercised through persuasion and excellent communication skills; it is based on a trusting relationship with the followers.

3) An effective leader needs to show sensitivity to being a positive role model, someone who sets the example for others to follow.

4) As is appropriate for any health and caring profession, leadership should also be humanistic; that is, leaders should act in ways that stress individuals' dignity and worth.

□ Being a good leader takes thought, care, insight, commitment, and energy. The leader demonstrates caring toward co-workers and clients.

Areas of Leadership

□ Leadership should be approached like any other new role or skill: slowly and carefully.

□ With each experience, growth occurs and leadership is strengthened.

□ 1) Patient Care Coordination

□ 2) Employee Responsibilities

□ 3) Nursing Care Delivery Models

□ 4) Delegating Nursing Care

□ 5) Nursing Department

□ 6) Employing Institution or Agency

Support for Leadership Training

Nurses intent on developing their leadership ability have multiple resources available to them—

□ Mentorship,

□ Preceptorship,

□ Participation in professional organizations, and

□ Continuing education.

FOLLOWERSHIP

□ Although the importance of leaders and followers working together in order to realize a vision are being acknowledged, but the literature typically pays little attention to the concept of followership, and there are no "theories" of followership.

□ Perhaps one of the earliest discussions of followership was presented by Kelley (1992, 1998), who outlined four types of followers: sheep, "yes" people, alienated followers, and effective or exemplary followers.

FOLLOWERSHIP CONT'D

□ Sheep are passive individuals who comply with whatever the

leader or manager directs but are not actively engaged in the work of the group.

- “Yes” people, in comparison, are actively involved in the group’s work and eagerly support the leader; they do not, however, initiate ideas or think for themselves.

- Alienated followers do think for themselves and often are critical of what the leader is doing; they do not, however, share those ideas openly, they seem disengaged, and they “rarely invest time or energy to suggest alternative solutions or other approaches”

- The individuals who are engaged, suggest new ideas, share criticisms with the leader, and invest time and energy in the work of the group are referred to as effective or exemplary followers.

FOLLOWERSHIP CONT'D

- Pittman, Rosenbach, and Potter (1998) also described four types of followers: subordinates, contributors, politicians, and partners.

- Subordinates are similar to Kelley’s “sheep,” doing what they are told but not actively involved.

- Contributors are like Kelley’s “yes people,” supportive, involved, and doing a good job, but not willing to challenge the ideas of the leader.

- Politicians are willing to give honest feedback and support the leader, but they may neglect the job and have poor performance levels.

- Like Kelley’s effective or exemplary followers, the partners described by Pittman, et al. (1998) are highly involved, perform at a high level, promote positive relationships within the group, and are seen as “leaders-in-waiting”

FOLLOWERSHIP CONT'D

- Because leaders cannot be leaders unless they have followers, the role of the follower is extremely important in any discussion of leadership.

- In addition, the characteristics that describe effective/exemplary followers or partners are quite similar to those outlined for effective leaders themselves.

- Effective in the role of followership are independent, critical thinkers, innovative, actively engaged, able and willing to think for themselves, willing to assume ownership, selfstarters, and able and willing to give honest feedback and constructive criticism

Guidelines for Delegating Nursing Care

- Delegation involves the transfer of responsibility for the

performance of an activity to another individual while retaining accountability for the outcome.

Before delegating any nursing intervention, a number of factors should be considered:

- (1) the patient's condition,
- (2) the complexity of the activity,
- (3) the potential for harm,
- (4) the degree of problem solving and innovation necessary,
- (5) the level of interaction required with the patient,
- (6) capabilities of the NAP, and
- (7) the availability of professional staff to accomplish the unit workload

Guidelines for Delegating Nursing Care 2

Nursing care or tasks that should never be delegated except to another RN include :

- The initial and ongoing nursing assessment of the patient and his or her nursing care needs
- The determination of the nursing diagnosis, nursing care plan, evaluation of the patient's progress in relation to care plan, and evaluation of the nursing care delivered to the patient
- The supervision and education of nursing personnel; patient teaching that requires an assessment of the patient and his or her education needs
- Any other nursing intervention that requires professional nursing knowledge, judgment, and/or skill.

Rights of Delegation

- ☐ Right Task: One that is delegable for a specific patient
- ☐ Right Circumstances: Appropriate patient setting, available resources, and other relevant factors considered
- ☐ Right Person: Right person is delegating the right task to the right person to be performed on the right person (patient)
- ☐ Right Direction/Communication: Clear, concise description of the task, including its objective, limits, and expectations
- ☐ Right Supervision: Appropriate monitoring, evaluation, intervention as needed, and feedback

Assignment

- ☐ Make comparisons between leaders and managers.
- ☐ Differentiate between transactional and

transformational leaders.

□ Compare the autocratic, democratic, Laissez-Faire and bureaucratic leaders on these headings: Degree of control, Leader activity level, Assumption of responsibility, Output of the group, and Efficiency.

□ Why is delegation necessary in nursing?
(Importance of delegation)

Paper 2 of 4 — MANAGEMENT OF NURSING SERVICES (NSC 413)

Level 500 • First Semester • Handout

MANAGEMENT OF NURSING SERVICES (NSC 413)

PLANNING

BY: MRS MONISOLA OMISHAKIN

INTRODUCTION

□ Planning is the work the manager does to predetermine a course of action, that involves- (a) Establishing Purpose – the work managers do to discover a vision of the future; (b) Setting Goals – the work managers do to determine the end results to be accomplished; and (c) Creating Plans – the work of sequencing tasks to accomplish goals.

□ Thus planning is an activity where goals are set, environmental studies are made, planning premises are considered and a plan is chalked out.

PLANNING

□ Different management scientists have defined the 'planning' differently.

□ Some definitions are as broad as – "Planning is anticipating" – Earl Strong.

□ Some recognize it as a process, like – "Planning is the thinking process, the organized forecast, the vision based on fact and experience that is required for intelligent action." – Alfred and Beaty.

□ Some view it in the form of outcomes of planning, like – "Management planning involves the development of forecasts, objectives, policies, programs, procedures, schedules, and budgets" – Louis A. Allen.

PLANNING CONT'D

□ When an organization wants to reach a goal, it makes a plan to reach it.

□ The plan is a future course of action. To decide this future course of action, the manager has to

anticipate future and take into account the organization's capability with regard to the resources required to implement the plan.

□ This plan should clearly lay out action sequences to be performed.

□ After putting the plan into action, the management keeps a track of it and make adjustments in it as and when required under the given circumstances.

□ Organizational plans generally take the shape of policies, procedures, budgets, programs, etc.

CHARACTERISTICS OF PLANNING

□ 1. Planning is the most basic of all managerial functions – Planning precedes all executory functions like organizing, directing, staffing, and controlling.

□ 2. Planning preconceives an objective – Every plan specifies the objectives to be attained in future and steps necessary to reach them.

□ 3. Planning is an intellectual activity- It involves vision and foresightedness to decide the things to be done in future.

□ 4. Planning must involve the future – It bridges the gap between where we are and where we want to go.

CHARACTERISTICS OF PLANNING CONT'D

□ 5. Planning is all pervasive – It is required in every managerial function and at every level.

□ 6. Planning is a continuous activity – It is never ending activity of a manager. Planning is always tentative and subject to revision and amendment, as new facts become known.

□ 8. Planning is Flexible: Planning is based on a forecast of future events. Since future is uncertain, plans should be reasonably flexible. When market conditions change, planners have to make necessary changes in the existing plans.

□ 9. Planning is an Integrated Process: Plans are structured in a logical way wherein every lower-level plan serves as a means to accomplish higher level plans.

PURPOSES OF PLANNING

- 1. Planning focuses attention on objectives: Planning directs explicit attention to objectives and priorities, and encourages individuals and organizations to focus on relevant results rather than on endless activities.
- 2. Planning establishes coordinated effort: It gives direction to managers and non-managers alike. When employees know where the organization is going and what they must contribute to reach the objective, they can coordinate their activities, cooperate with each other, and work in teams. Without planning, departments could be working at cross-purposes and preventing the organization from moving efficiently towards its objectives.
- 3. Planning reduces uncertainty: Planning forces managers to look ahead, anticipate change, consider the impact of change, and develop appropriate responses. It also clarifies the consequences of actions managers might take in response to change.

PURPOSES OF PLANNING CONT'D

- 4. Planning provides a means for organizations to cope with changes in their environment.
- 5. Planning reduces overlapping and wasteful activities: Planning beforehand is likely to pinpoint waste and redundancy. Furthermore, when means and ends are clear, efficiencies become obvious.
- 6. Planning establishes objectives or standards that are used in controlling: In the controlling function, we compare actual performance against the objectives, identify any significant deviations, and take the necessary corrective action. Without planning, there would be no way to control.
- Comprehensive planning is an integrative activity that seeks to maximize the total effectiveness of an organization as a system in accordance with its goals.

BASIC STEPS IN PLANNING PROCESS

- Step 1- Identify Need for Planning: First of all need for making plans should be identified clearly. The managers should be sensitive enough to judge which area needs planning. For example- falling sales of the company may alarm the managers to review and

plan its sales function.

□ Step 2- Establish a Goal or Set of Goals: Next step is to decide what exactly the organization or sub-unit wants or needs in order to accomplish organizational purpose. For example- the goal of the marketing unit may be to reach a particular quantum of turnover in the given time. Identifying priorities and being specific about them enable organizations to focus their resources effectively.

BASIC STEPS IN PLANNING PROCESS CONT'D

□ Step 3- Define the Present Situation: How far is the organization or the subunit from its goals? What resources are available for reaching the goals? Only after the current state of affairs is analyzed can plans be drawn up to chart further progress. Open lines of communication within the organization and between its sub-units provide the information—especially financial and statistical data—necessary for this third stage.

□ Step 4- Identify the Aids and Barriers to the Goals: What factors in the internal and external environments can help the organization reach its goals? What factors might create problems? Planning requires an intimate understanding of present resources (human, material, and financial), core competencies, and actual obstacles with respect to the business unit, all of which describe the current condition.

BASIC STEPS IN PLANNING PROCESS CONT'D

□ Step 5- Develop a Plan or Set of Actions for Reaching the Goal(s): Each Goal must be broken down into an Action Plan which has four parts to it-

□ (1) The WHAT - This involves tasks- although goals clarify what is to be done in organization, each goal must be broken down into specific tasks that will help to accomplish the goal.

□ (2) The WHO - this involves delegating - the work a leader does to identify the capabilities of each member of the team and delegate tasks to them.

□ (3) The WHEN - this involves scheduling- the work of putting a time factor on our programme and inserting the calendar into the programme with dates and time durations.

□ (4) The WITH WHAT - this involves budgeting - the application of resources (personnel, time and equipment) to help achieve the goal.

□ Step 6- Monitor the Progress of Plan: After the plan is set into

motion, next task is to monitor its progress. Is it leading the organization in the same direction as was perceived while planning? If not, what could be the reason?, can any change make the things better?, if yes, the planning process again starts. Monitoring is a continuous process, and is required to be done at each stage of implementation of plans, and so is the planning process.

PRINCIPLES OF PLANNING

- 1. Principle of Contribution to Objectives: Planning is meant to achieve the objectives of the organization. Thus it should focus attention to objectives and direct all efforts and resources to their advancement.
- 2. Principle of Primacy of Planning: There are different functions of management, but planning must hold the primacy and thus occupy first place and precedes all other functions of management.
- 3. Principle of Planning Premises: Planning is always based on some premises which includes forecasts of future environment, current information of environment and organization, values and assumptions of planners and owners of organization.

PRINCIPLES OF PLANNING CONT'D

- 4. Principle of Limiting Factors: Planning must take the limiting factors (manpower, money, machines, materials and management) into account by concentrating on them when developing alternative plans, strategies, policies, procedures and standards.
- 5. Principle of Commitment: An organization should plan in the future for a period of time sufficient to fulfil the commitments of the organization. This principle helps in determining the length of the planning period. It suggests that the time period covered by planning should be related to the commitments of the organization.

PRINCIPLES OF PLANNING CONT'D

- 6. Principle of Flexibility: Flexibility in the plans is required to cope with the uncertainties of the environment. However, flexibility entails costs. Thus, degree of flexibility in a plan should be decided after weighing the costs and probable

benefits of the given flexibility.

□ 7. Principle of Navigational Change: It is the duty of the navigator to check constantly whether his ship is following the right direction in the vast ocean to reach the destination as scheduled. The navigator changes the path of ship in case it is not going on the right path. In the same way, a manager should check his plans to ensure that these are progressing as required. He should change the direction of his plans if he faces unexpected events.

TYPES OF PLANS

□ As the plans are all pervasive, they are made at every level of organization with different purposes and perspectives.

Accordingly they may take various shapes and stand differently in the hierarchy of importance.

□ The most popular ways to describe organizational plans are by their:

- 1. Coverage – Strategic, tactical, and operational,
- 2. Time frame – Short and long term,
- 3. Specificity – Specific versus directional,
- 4. Frequency of use – Single use and standing.

□ Note that these planning classifications aren't independent. For instance, short-and long- term plans are closely related to strategic and operational ones. And single-use plans typically are strategic, long term, and directional.

STRATEGIC, TACTICAL AND OPERATIONAL PLANS

□ a. Strategic Plans:

□ Strategic plans are designed to meet the broad objectives of the organization – to implement the mission that provides the unique reason for organization's existence. They are set at the top managerial level, and are meant to guide the whole organization.

□ An organization's strategic plan is the starting point for planning. The aim of strategic planning is to help a company select and organize its businesses in a way that would keep the company healthy in spite of unexpected upsets occurring in any of its specific businesses or product lines.

□ For example- in order to deal with uncertainties of raw

material availability, a company's strategic plan may purport to acquire its own facilities for generating raw material. Strategic plan serves as a guide to the development of sound sub plans to accomplish the organizational objectives.

STRATEGIC, TACTICAL AND OPERATIONAL PLANS CONT'D

□ b. Tactical Plans: Top level managers set the strategies that an organization should focus to achieve organizational goals. Examples of strategies include set-up a plant to generate raw material for the organization's manufacturing activities, explore North-East market, and likewise. Middle managers interpret these strategies and develop tactical plans for their departments that follow strategies in order to contribute to the organizational goals.

□ In order to develop tactical plans, middle management needs detail reports (financial, operational, market, external environment). Tactical plans have shorter time frames and narrower scopes than strategic plans.

STRATEGIC, TACTICAL AND OPERATIONAL PLANS CONT'D

□ c. Operational Plans: The supervisor interprets the strategic and tactical management plans as they apply to his unit. This way, he makes operational plans to support tactical plans. These plans provide the details of how the strategic plans will be accomplished.

□ Examples of planning by supervisors include scheduling the work of employees and identifying needs for staff and resources to meet future changes. Operating plans tend to be repetitive and inflexible over the short run. Change comes only when it is obvious that plans and specific action steps are not working.

□ There are two main type of operational plans – Single use plans which are developed to achieve specific purposes and dissolved when these have been accomplished; standing plans are standardized approaches for handling recurring and predictable situations.

2. SHORT-RANGE AND LONG-RANGE PLANS

□ Time is an important factor in planning. The time period

covered by planning should preferably include sufficient time to fulfill the managerial commitments involved.

□ Generally a short range planning (SRP) means a plan for one or two years and long range planning (LRP) means a plan for three to five years or more. Though this division may be considered as arbitrary, but it may have a general acceptability. This period of course, may vary according to the nature and size of business.

□ When a concern requires long gestation period, it is natural that the long range planning may cover a longer period than five years.

□ However, whatever the period of planning, it should not be too rigid. It should rather be flexible to meet the unknown factors of the future. If a concern adopts both short-term and long-term planning, the short-term planning should fit in with long-term planning.

3. SPECIFIC AND DIRECTIONAL PLANS

□ Specific plans are established to achieve a specific purpose and dissolves when the purpose is accomplished. For example- a manager who seeks to increase his firm's sales by 20 per cent over a given twelve-month period might establish specific procedures, budget allocations, and schedules of activities to reach that objective. These represent specific plans.

□ Directional plans identify general guidelines. They provide focus but do not lock managers into specific objectives or courses of action. Instead of following a specific plan to cut costs by 4 per cent and increase revenues by 6 per cent in the next six months, a directional plan might shoot for improving corporate profits by 5 to 10 per cent every year.

□ Intuitively, it seems right that specific plans would be preferable to directional or loosely guided plans, because they have clearly defined objectives. There is no ambiguity, no problem with misunderstandings. However, in certain circumstances, like in case of fast changing environment, directional plans provide the flexibility required to cope with the changing situations.

4. SINGLE USE AND STANDING PLANS

□ A single-use plan is a one-time plan specifically designed to meet the needs of a unique situation and created in response to non-programmed decisions that managers make.

□ In contrast, standing plans are ongoing plans that provide guidance for activities repeatedly performed in the organization. Standing plans are created in response to programmed decisions that managers make and include the policies, rules, and procedures.

SINGLE USE

□ a. Programs: A program covers a □ c. Budgets: Budgets are relatively large set of activities. The statements of financial program shows-

□ (1) the major steps required to resources set aside for specific reach an objective, activities in a given period of

□ (2) the organization unit or member time. They are primarily devices responsible for each step, and to control an organization's

□ (3) the order and timing of each activities and so are important step. The program may be components of programs and accompanied by a budget or a set of budgets for the activities required. projects. Budgets itemize

□ b. Projects: Projects are the smaller income as well as expenditures and separate portions of programs. and thus provide targets for such Each project has limited scope and activities as sales, departmental distinct directives concerning expenses, or new investments. assignments and time. In the warehouse example, typical projects □ Managers often use budget might include the preparation of development as the process by layouts, a report on labour availability, and recommendations which decisions are made to for transferring stock from existing commit resources to various facilities to the new installation. alternative courses of action. In Each project will become the this sense, budgets can be responsibility of designated personnel who will be given specific considered single-use plans in resources and deadlines. their own right.

STANDING PLANS

□ a. Policies: A policy is a general statement designed to guide employees' actions in recurring situations. It establishes broad limits, provides direction, but permits some initiative and discretion on the part of the supervisor. Thus, policies are guidelines. Some policies deal with very important matters, like those requiring strict sanitary conditions where food or drugs

are produced or packaged. Others may be concerned with relatively minor issues, such as the way employees dress.

□ b. Procedures: A procedure is a sequence of steps or operations describing how to carry out an activity. It is more specific than a policy and establishes a customary way of handling a recurring activity.

□ c. Rules: A rule is an established guide for conduct. Rules include definite things to do and not to do. There are no exceptions to the rules. An example of a rule is “No Smoking.” Managers frequently use rules when they confront a well-structured problem because they are simple to follow and ensure consistency. For example- rules about lateness and absenteeism permit supervisors to make disciplinary decisions rapidly and with a relatively high degree of fairness.

DISTINCTION BETWEEN POLICIES AND PROCEDURES

□ The terms ‘policies’ and ‘procedures’ themselves explain their scope. However, following are some criteria to differentiate them:

- 1. Basis of Formulation: Policies are formulated on the basis of objectives of organization. Procedures are determined on the basis of work to be carried out for achieving those objectives.
- 2. Level of Formulation: Policies are generally formulated by top management. Procedures are laid down at the lower level of management.
- 3. Comparative Status: In the hierarchy of plans, policies occupy higher place. Procedures occupy lower place as compared to policies.
- 4. Focus Area: Policies deal with functional aspects of management. Procedures deal with operational or administrative aspect of management.
- 5. Detailing of Instructions: Policies are general statements which provide guidance to managers in decision making. Procedures are regarded as a detailed guidelines for performing activities.
- 6. Rigidity: Policies are broad based providing a scope to the manager for using his skill and experience. Procedures are rigid and leave no scope for personal judgment.
- 7. Need of Skill in Application: Policies encourage initiative of the managers, because they get opportunity for using their skill, experience and knowledge and personal judgment. Procedures do not help in developing initiative among the employees, because they do not leave scope for personal judgment.

FEATURES OF A GOOD PLAN

□ A sound plan has the following features:

- 1. Integration: A good plan integrates short-term requirements of the firm with the long- term requirements. Plans (short-term and long-term) should be oriented towards the overall

organizational goals.

□ 2. Market Research: Planners should conduct market research before they frame plans. “What potential customers say they are going to do and what they end up doing may be two different things?” Plans should forecast market requirements through a well conducted market research.

□ 3. Economy/Financial constraints: A plan which relies too heavily on financial budgets may turn out to be a failure if the revenues do not arise as expected. Once the budgetary balance is disturbed, subsequent operations may get affected as plan is a sequence of cause and effect relationships. Future costs and benefits should be carefully analyzed while designing organizational plans.

FEATURES OF A GOOD PLAN CONT'D

□ 4. Co-ordination: A sound plan should co-ordinate working of all the functional areas. If functional plans (or departmental plans) are not synchronized with organizational plans, organization will fail to achieve its goals.

□ 5. Consistent: Plans should be followed for a fairly long of time. Frequent modifications/ alterations in plans by higher authorities can make their implementation ineffective at lower levels.

□ 6. Flexible: Though consistent, plans should adjust (flexible) to environmental changes.

□ 7. Acceptable: Best laid plans may turn out to be failures if they are not implemented effectively. Plans should be acceptable to those who frame them and also to those who implement them.

FEATURES OF A GOOD PLAN CONT'D

□ 8. Participative: The acceptability of plans increases if followers participate in the planning process. Many organizations follow the principle of participation in planning. Managers invite ideas/suggestions from people of different departments at different levels and finalize the plans.

□ 9. Clear Objectives: Plans should be oriented towards objectives. Clear, specific and attainable objectives result in sound and effective plans.

□ 10. Based on Planning Premises: Since plans achieve a goal in future, they should be based on accurate forecasts and predictions about future events. Planning premises provide a

basis for making future oriented plans.

□ 11. Effective Communication System: Framing and implementing plans requires interaction of managers at all levels with people inside and outside the organization. A sound plan is based on well-designed communication system present in the organization.

RELATIONSHIP BETWEEN PLANNING AND CONTROL

□ 1. Planning identifies actions and controlling ensures that actions are carried out.

□ 2. Poor control system is followed by failure of plans and effective control system reinforces the plans.

□ 3. Controlling helps in revising the plans or abandoning them altogether and making fresh plans.

□ 4. Framing plans presumes existence of a well-designed control system and controlling function presumes existence of plans achievable within the constraints of human and non-human resources.

IMPORTANCE OF PLANNING

□ 1. Achievement of □ 6. Optimum utilization of organizational objectives resources/efficiency of operations

□ 2. Fulfillment of □ 7. Development of organizational managers commitments:

□ 8. Promotes innovation/creativity

□ 3. Facilitates decision-making: □ 9. Basis for control

□ 10. Reduces risk

□ 4. Provides stability to □ 11. Morale boost up organizations

□ 12. Facilitates delegation

□ 5. Overall view of the organization/coordination

BARRIERS TO PLANNING

□ 1. Inappropriate Goals: Goal setting is the essential first step in planning. Managers who are unable to set meaningful goals will be unable to make meaningful plans. There may

be several reasons why inappropriate goals may be set, like lack of organizational and environmental knowledge.

□ 2. A Dynamic and Complex Environment:

Planning essentially requires forecasting. And forecasting needs assessment of future environment. A dynamic and complex environment makes this task very difficult and prone to errors.

BARRIERS TO PLANNING CONT'D

□ 3. Reluctance to Establish Goals: There are a number of reasons why some managers hesitate – or fails entirely – to set goals for their organization or subunits. Some of these are— Unwillingness to give up alternative goals, Fear of failure, Lack of information, Lack of confidence.

□ 4. Resistance to Change: At times there is a general reluctance in organization members to accept planning and plans because of the changes they bring.

□ 5. Other Constraints: There may be many other constraints like, attitude of owner- managers towards planning, lack of consensus on planning issues because of communication gaps, managerial conflicts and lack of understanding the real perspective of the organization.

METHODS FOR OVERCOMING BARRIERS TO PLANNING

□ 1. Understanding the Organizational Perspective Clearly: The purpose of an organization should be very clear and be communicated clearly to every member of the organization.

□ 2. Understanding the Purposes and Process and Plans: Provide more information to employees about plans and their probable consequences so that they will understand the need for change, the expected benefits, and what is required for effective implementation.

□ 3. Setting Realistic Goals: Fear of failure and lack of confidence are also reduced by setting realistic goals and achieving them. Providing training and guidance

in ways to achieve such goals is one important step.

METHODS FOR OVERCOMING BARRIERS TO PLANNING CONT'D

- 4. Develop a Good Information Network: Sound information base helps in promoting effective planning.
- 5. Communication: Effective and well communicative systems of planning help in bringing consensus on planning issues and do away with the misconceptions.
- 6. Participation: Involve employees and other concerned groups, including stakeholders, in the planning process. One particularly useful technique for managing goal setting and planning is formal goal setting, a process of collaborative goal setting and planning.
- The basic need to plan effectively is conscious and coordinated efforts towards it. The barriers, once identified can be tackled.

Paper 3 of 4 — MANAGEMENT OF NURSING SERVICES (NSC 413)

Level 500 • First Semester • Handout

MANAGEMENT OF NURSING

SERVICES (NSC 413)

MANAGEMENT THEORIES

BY

MRS MONISOLA OMISHAKIN

MANAGEMENT

- The essence of management is getting work done through others.
- The classic definition of management is Henri Fayol's 1916 list of managerial tasks: planning, organizing, commanding, coordinating, and controlling the work of a group of employees.
- But Mintzberg (1989) argued that managers really do whatever is needed to make sure that employees do their work and do it well.
- Lombardi (2001) points out that two-thirds of a manager's time is spent on people problems.
- The rest is taken up by budget work, going to meetings, preparing reports, and other administrative tasks.

MANAGEMENT THEORIES

Three categories of traditional management theory have evolved:

General

Scientific Bureaucratic

administrative

management management.

management

SCIENTIFIC MANAGEMENT

- Almost 100 years ago, Frederick Taylor argued that most jobs could be done more efficiently if they were analyzed thoroughly.
- With a well-designed task and enough incentive to get the work done, workers could be more productive. For example, Taylor promoted the concept of paying people by the piece instead of by the hour.
- In health care, the equivalent would be by the number of patients bathed or visited at home rather than by the

number of hours worked.

□ This would create an incentive to get the most work done in the least amount of time.

□ Taylorism stresses that there is a best way to do a job.

Usually, this is also the fastest way to do the job

SCIENTIFIC MANAGEMENT 2

□ The work is analyzed to improve efficiency.

□ In health care, for example, there has been much discussion about the time it takes to bring patients to radiology or to physical therapy versus bringing the technician or therapist to the patient.

□ Eliminating excess staff or increasing the productivity of remaining employees is also based on this kind of thinking.

□ Nurse managers who use the principles of scientific management will pay particular attention to the type of assessments and treatments done on the unit, the equipment needed to do this efficiently, and the strategies that would facilitate efficient accomplishment of these tasks.

□ Typically, these nurse managers keep careful records of the amount of work accomplished and reward those who accomplish the most.

GENERAL ADMINISTRATIVE THEORY

□ Henri Fayol (1841–1925) was a Frenchman who is remembered for the development of general administrative theory.

□ He developed his management strategies in the mining industry and was writing at about the same time as Taylor.

□ Management, according to Fayol's work, includes five overriding concepts:

□ (1) prevoyance, or the anticipation of the future and the development of a plan of action to deal with it;

□ (2) organization of people and materials;

□ (3) command of the activity among personnel;

□ (4) coordination of the parts of the organization into a unified whole; and

□ (5) control through application of rules and procedures.

□ In order for an organization to be productive, leaders must participate actively in all five of these areas.

□ Fayol is remembered for his 14 principles of management, which he felt supported the accomplishment of the overriding concepts.

GENERAL ADMINISTRATIVE THEORY 2

□ These principles introduced some ideas that continue to be

used.

- For example, in the 21st century, hospital personnel departments continue to have a pay scale that strives to provide fair remuneration based on educational preparation and years of experience.
- Every organization strives to retain its staff because of the cost of recruiting, training, and orienting new employees.
- The development of “esprit de corps,” or team spirit, continues to be important in today’s workplace.
- Teamwork remains essential to providing optimal patient care, and high morale is conducive to the levels of collaboration and teamwork that are required in the complex health-care environment.
- Patient care is delivered by a collaborative team of knowledge workers including nurses, physicians, and therapists from a variety of disciplines, all of whom are necessary to the outcome of optimal patient care.

ASSIGNMENT

- Describe the 14 principles of management according to Henri Fayol and its implication for Nursing.
- Submit 7th November, 2022.

BUREAUCRATIC MANAGEMENT

- Max Weber (1846–1920) was a German sociologist who developed what was known as the “ideal bureaucracy.”
- The ideal bureaucracy includes the concepts of division of labour, authority hierarchy, formal selection, formal rules and regulations, impersonality, and career orientation.
- He recognized that it would be impossible for people to be completely impersonal in their relationships at work, but he believed that impersonality would be optimal and would remove favouritism.
- Weber believed that the more impersonal, rational, and regulated the work environment, the more likely the employees were to be treated fairly, and the more likely the organization was to reach its objectives.

BUREAUCRATIC MANAGEMENT 2

- Weber focused on what it was that made people respond to authority.

- He perceived that only through concentrating power in the hands of a few people in a hierarchical structure could an organization be managed effectively and efficiently.
- While he did not necessarily agree that bureaucracy was the best strategy, because it removed autonomy from the individual, he believed it was the only way to assure the overall success of an organization
- During the early 20th century when Taylor, Fayol, and Weber developed these approaches to management, the worldview was still based upon 17th-century science.

TRADITIONAL MANAGEMENT

- In general, traditional management styles have their advantages and disadvantages.
- The prime advantage is that they enhance the organization and efficiency of industry.
- The disadvantages of traditional management include rigid rules, top-down decision making, and authoritarianism.
- In other words, traditional management theory created an environment that was less optimal from a humanistic perspective

HUMAN RELATIONS-BASED MANAGEMENT

- McGregor's theories X and Y provide a good example of the difference between scientific management and human relations-based management.
- Theory X, said McGregor (1960), reflects a common attitude among managers that most people do not want to work very hard and that the manager's job is to make sure that they do work hard.
- To accomplish this, according to Theory X, a manager needs to employ strict rules, constant supervision, and the threat of punishment (reprimands, withheld raises, and threats of job loss) to create industrious, conscientious workers.

HUMAN RELATIONS-BASED MANAGEMENT 2

- Theory Y, which McGregor preferred, is the opposite viewpoint.
- Theory Y managers believe that the work itself can be motivating and that people will work hard if their managers provide a supportive environment.

□ A Theory Y manager emphasizes guidance rather than control, development rather than close supervision, and reward rather than punishment.

□ A Theory Y nurse manager is concerned with keeping employee morale as high as possible, assuming that satisfied, motivated employees will do the best work.

□ Employees' attitudes, opinions, hopes, and fears are important to this type of nurse manager.

THEORY Z: JAPANESE MANAGEMENT STYLE

□ In 1981 William G. Ouchi wrote a book on Japanese management style, entitled Theory Z. In this book he discussed the management methodologies used by Japanese corporations.

□ This approach to management relied on principles that were diametrically opposed to those used in businesses in the West, including America, England, and Europe.

□ Employment in the Japanese corporation is described as being lifelong, dependent upon the development of consensus, collaborative work, incentives for group work, and pride in the product or service being developed or provided.

THEORY Z 2

□ In Japan, the word kaizen refers to the principle of encouraging all people in the organization to contribute improvement ideas on a biweekly basis.

□ Organizational growth has been shown to be directly related to innovation.

□ The more leadership encourages participation and ownership among the employees, the more productive the organization becomes.

□ Ouchi (1981) discusses the importance of encouraging group contributions.

□ In Japan, individuals rarely desire personal recognition because they believe that nothing is possible without everyone's contributions.

21ST-CENTURY MANAGEMENT

THOUGHT

□ Management theory in the first decade of the 21st century is influenced by a new worldview, which has, once again, had its roots in the physical

sciences.

□ Managers are beginning to recognize that the direct cause and effect relationships, to which they held in the past, frequently do not exist in reality.

□ Additionally, management theories are being communicated in more complex systems and in professional systems, in contrast to the earlier management theories that began to emerge during the manufacturing environment of the 17th century.

21ST-CENTURY MANAGEMENT THOUGHT 2

□ Chaos theory and complexity theory, which have emerged from quantum physics, now underscore our understanding and interpretation of the work people do in organizations.

□ Hock (2000) has even coined a new term for management based in complexity science: chaordic (kay-ordic).

□ The word borrows the first syllable of the word chaos and the word order.

□ He defines the term chaord as “any self-organizing, self-governing, adaptive, nonlinear, complex organism, organization, community or system, whether physical, biological or social, the behaviour of which blends characteristics of both chaos and order”

□ Organizations have elements of both chaos and order, with innovation and progress occurring “at the edge of chaos.”

COMPLEXITY THEORY

□ Complexity science “is not a single theory.

□ It is the study of complex adaptive systems—the patterns of relationships within them, how they are sustained, how they self-organize, and how outcomes emerge.

□ Within the science there are many theories and concepts.

Complexity science is highly interdisciplinary including biologists, anthropologists, economists, sociologists, management theorists and many others in a quest to answer some fundamental questions about living, adaptable, changeable systems”

□ The idea that systems in nature are self-organizing lends support for the knowledge worker supported by Drucker in that

individuals within an organization can build a better system, bringing order out of chaos, when allowed to self-organize.

□ Ideas from complexity theory, such as chunking, attractors, self-organization, distributed control, and leveraging incremental changes, can be used in health-care organizations.

COMPLEXITY THEORY 2

□ Employees, managers, and organizations are rapidly changing and becoming more flexible in their interactions with each other.

□ Unstated in a job description is the expectation that the employee engage in the critical thinking, innovation, and interpersonal relationships required to accomplish the goals and objectives of the position.

□ Peter Drucker revised his thinking to recognize that in the 21st century employees are actually “knowledge workers” who necessarily know more about their area of responsibility than do their managers.

□ The knowledge worker must be able to make decisions and implement strategies that work; these changes can be made more effectively and efficiently at the point of contact of the worker with the environment than by management removed by several layers.

COMPLEXITY THEORY 3

□ Complexity theory does not disregard previous management theories; instead, it borrows concepts from many theories, modifying them as part of the evolutionary process.

□ Management using complexity theory is neither totally mechanistic nor behaviourist.

□ Instead, it is a new, ever-changing process.

□ The manager has much in common with the artistic director of a ballet production who choreographs the dance moves, selects the music, and plans lighting and scenery.

□ During the production, however, the dancers make the magic of the movements come to life.

□ The entire performance is much more than the sum of the individual movements and roles.

□ The application of complexity theory, with its reliance on self-organization, offers solutions for nursing and today’s health-care organizations.

□ Strategies for applying this new science will continue to evolve.

Paper 4 of 4 — MANAGEMENT OF NURSING SERVICES (NSC 413)- MANAGEMENT PROCESS

Level 500 • First Semester • Handout

MANAGEMENT OF NURSING

SERVICES (NSC 413)

MANAGEMENT PROCESS

BY

MRS MONISOLA OMISHAKIN

MANAGEMENT PROCESS

□ Management is the art of getting things done through and with people in formally organized groups.

□ We manage an organization to achieve certain objectives and goals.

□ For this purpose, the manager performs some fundamental functions. They are called managerial functions

MANAGEMENT PROCESS CONT'D

□ Henry Fayol, the father of principles of management, has classified managerial functions as follows:

(a) Planning, including forecasting, (b) Organizing, (c) Commanding, (d) Coordinating, and (e) Controlling.

□ Gulick and Urwik have classified management functions into seven. They coined the word 'PODSCORB' to describe the functions of management.

□ Each letter of this word denotes the initial letter of management functions, namely- (a) Planning, (b) Organizing, (c) Directing, (d) Staffing, (e) Coordinating, (f) Reporting, and (g) Budgeting.

□ Koontz and O'Donnell have classified management functions into five. They have stated that "the most useful method of classifying managerial functions is to group them around the activities of planning, organizing, staffing, directing, and controlling." Thus, according to them, there are five functions of management.

MANAGEMENT PROCESS CONT'D

□ It is helpful to think of these managerial functions as a continuous circular movement.

□ They flow into each other and at times, it is difficult to know where the one ends and the other begins.

□ However, the following comprehensive classification can be given of various managerial functions:

- 1. Planning,
- 2. Organizing,
- 3. Staffing,
- 4. Directing –
 - (a) Leadership,
 - (b) Communication,
 - (c) Motivation, and
 - (d) Supervision.
- 5. Coordinating, and
- 6. Controlling.

PLANNING

□ Planning is concerned with the determination of the objectives to be achieved and the course of action to be followed to achieve them.

□ Before starting any action, one has to decide how the work will be performed and where and how it has to be performed. Thus, planning implies decision-making as to what is to be done, how it is to be done, when it is to be done and by whom it is to be done.

□ Planning helps in achieving the objectives efficiently and effectively.

□ Planning involves selecting of objectives and strategies, policies and programmes and procedures for achieving them.

PLANNING CONT'D

□ Planning function is performed by managers at every level because planning may either be for the entire enterprise or for any section or department thereof.

□ Planning pervades the entire gamut of managerial activity, and also it is continuous and never-ending.

□ While the managers at the top level devote more time on planning, the managers at the lower level follow the policies, programmes and procedures laid down by the top management.

□ For any business activity, planning is a prerequisite for doing anything and also to ensure the proper utilization of the resources of the business concern to achieve the desired goals.

□ Plans can be classified into standing plans and single-use

plans. Standing plans include objectives, policies, procedures, methods and rules and single-use plans include budgets, programmes, strategies and projects.

ORGANIZING

□ According to Fayol, “to organize a business is to provide it with everything useful to its functioning — raw materials, tools, capital and personnel.”

□ Thus, organizing involves bringing together the manpower and material resources for the achievement, of objectives laid down by the enterprise.

□ Organizing involves the following process:

□ (a) Determining and defining the activities involved in achieving the objectives laid down by the management;

□ (b) Grouping the activities in a logical pattern;

□ (c) Assigning the activities to specific positions and people; and

□ (d) Delegating authority to their positions and people so as to enable them to perform the activities assigned to them.

ORGANIZING CONT'D

□ Organizing function helps in increasing the efficiency of the enterprise. Further, by avoiding repetition and duplication of activities, it reduces the operation cost of the enterprise.

□ But organizing function can be useful to the enterprise only when there are clear and verifiable objectives and clear understanding of the activities needed to achieve the objectives and clear definition of the authority assigned to the managers at every level.

STAFFING

□ The function involves manning the positions created by organization process.

□ It is concerned with human resources of an organization.

□ In the words of Koontz and O'Donnel, “staffing is filling, and keeping filled, positions in the organization structure through defining work-force requirements/appraising, selecting, compensating and training.”

STAFFING CONT'D

- Thus, staffing consists of the following:
- (i) Manpower planning, i.e., assessing manpower requirements in terms of quantity and quality;
- (ii) Recruitment, selection and training;
- (iii) Placement of man power;
- (iv) Development, promotion, transfer and appraisal; and
- (v) Determination of employee remuneration.
- Every manager in an organization has to perform the staffing function in one form or the other, in order to get things done through others.
- But, it is decidedly a difficult managerial function as it concerns human beings whose behaviour and actions cannot be predicted, and that is why it has become a distinct and specialized branch of management.

DIRECTING

- Directing is one of the important functions of management and is the art and process of getting things done.
- While other functions like planning, organizing and staffing are merely preparations for doing the work, the directing function actually starts the work.
- Directing is concerned with actuating the members of the organization to work efficiently and effectively for the attainment of organization goals.
- Directing involves the manager telling the subordinates how they have to perform jobs assigned to them.
- It is concerned with guiding, supervising and motivating the subordinates for achieving the enterprise objectives.

DIRECTING CONT'D

- According to Joseph Massie, "Directing concerns the total manner in which a manager influences the action of his subordinates. It is the final action of a manager in getting others to act after all preparations have been completed."
- Directing consists of the following four sub-functions:
- a. Communication or issuing of orders and instructions to subordinates. A manager has to instruct the subordinates what to do, how to do it and when to do it.
- b. Guiding, energising and leading the subordinates to perform

the work systematically and also building up among workers confidence and zeal in the work to be performed.

- c. Inspiring the subordinates to do work with interest and enthusiasm for the accomplishment of the enterprise's objectives.
- d. Exercising supervision over the subordinates to ensure that the work done by them is in conformity with the objectives that are determined.

COORDINATION

- Co-ordination is one of the most important functions of management.
- It is essential to channelize the activities of various individuals in the organization for the achievement of common goals.
- Every department or section is given a target to be achieved and they should concentrate only on their work and should not bother about the work of other organs.
- It is left to the management to see that the work of different segments is going according to pre-determined targets and corrective measures have to be taken if there is any deviation.

COORDINATION CONT'D

- Co-ordination creates a team spirit and helps in achieving goals through collective efforts.
- It is the orderly arrangement of group effort to provide unity of action in the pursuit of common objectives.
- Dalton McFarland defines co-ordination as the, "process whereby an executive develops an orderly pattern of group effort among his subordinates and secures unity of action in the pursuit of common purposes."
- Co-ordination can be classified under two categories – (i) vertical and horizontal co-ordination, and (ii) internal and external co-ordination.

COORDINATION CONT'D

- Whereas vertical co-ordination is the co-ordination between different levels of management, the term horizontal co-ordination is used when co-ordination has to be achieved between departments of the same level of authority.

□ Co-ordination is internal when it is between different sections of the same concern and external when it is required with persons outside the organization.

□ Co-ordination is regarded as the very essence of management as in order to co-ordinate the activities of his subordinates, a manager has to perform all the other functions of management, viz., planning, organizing, staffing, directing and controlling.

CONTROLLING

□ Controlling can be defined as – “determining what is being accomplished, that is evaluating the performance, if necessary, applying corrective measures so that the performance takes place according to plans.”

□ Control is essential for achieving objectives of an enterprise. The planning of various activities does not ensure automatic implementation of policies.

□ Control is the process which enables management to get its policies implemented and take corrective actions if performance is not according to the pre-determined standards.

□ If planning is the beginning of the management process, controlling may be said to be the final stage. If planning is looking ahead, controlling is looking back.

□ Control is not possible without planning and planning is meaningless without control.

CONTROLLING CONT'D

□ Control is a line function and executives at various levels of management continuously assess the performance of their subordinates.

□ The main purpose of control is to see that the activity is achieving the desired results.

□ A control system, to be effective, must conform to the nature of activity, report deviations promptly, reflect organization structure, assure corrective action and be economical.

□ The process of controlling involves the following steps:

□ (i) Establishing standards of performance;

□ (ii) Measuring actual performance;

□ (iii) Comparing the actual performance with the standard;

□ (iv) Finding variances or deviations, if any; and

□ (v) Taking corrective action or measures.

INTER-RELATIONSHIP BETWEEN MANAGEMENT FUNCTIONS

□ Management is an integrated process consisting of a number of functions such as planning, organizing, staffing, directing, coordinating and controlling.

□ Every management has to perform these functions at one time or the other or at the same time. Though the functions have theoretically been independent from each other but practically they are overlapping and cannot be separated.

□ Each function is blended into the others and each affects the performance of other functions.

□ There is no rigid sequence of performing these functions. It is not necessary that organizing will be performed only after the planning function. No hard and fast sequence is followed for performing the functions.

□ It may be possible that some or all functions are performed at the same time. So various functions are inter-related and are dependent upon one another for improving their performance.